

Bendamustine: Belrapzo, Bendeka, Treanda, Vivimusta

Original Effective Date: 12/5/2022

Revision Date(s): 12/5/2022; 11/7/2023; 4/25/2024; 9/24/2024; 11/4/2024; 10/1/2025

Review Date: 10/1/2025

Policy type: Medical Necessity

Line of Business: All GEHA Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Preference	Requires Prior Auth	Drug Name	HCPCS Code	Description
Preferred	Yes	Treanda	J9033	Injection, bendamustine hydrochloride, 1 mg
Preferred	Yes	Bendamustine 505(b)(2) Dr Reddy's	J9999	Not otherwise classified, antineoplastic drugs
Non-preferred	Yes	Bendeka	J9034	Injection, bendamustine hcl (Bendeka), 1 mg
Non-preferred	Yes	Belrapzo	J9036	Injection, bendamustine hydrochloride, (Belrapzo/bendamustine), 1 mg
Non-Preferred	Yes	Vivimusta	J9056	Injection, bendamustine hydrochloride (Vivimusta), 1 mg

- Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents (or their 505(b)(2) equivalents) prior to approval of a non-preferred product, when clinically appropriate.
- Preferred product requirements apply to all requests, both new and reauthorizations.
- Step Therapy applies to all overlapping compendia supported indications/regimens.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

- National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; **OR**
- United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; **OR**
- Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:
 - Clinical Pharmacology (Strong For); **OR**
 - Wolters Kluwer Lexi-Drugs® (Level A); **OR**

- Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:
 - American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;
 - **Meeting abstracts and case reports are excluded from consideration;**
- Non-standard protocols may be approved based on unique clinical circumstances;
- Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network® (NCCN), or indication specific peer-reviewed literature;
- In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing

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Belrapzo	J9036	Injection, bendamustine hydrochloride, (Belrapzo/bendamustine), 1 mg
Vivimusta	J9056	Injection, bendamustine hydrochloride (Vivimusta), 1 mg
Bendamustine 505(b)(2) Dr Reddy's	J9999	Not otherwise classified, antineoplastic drugs

References

1. Belrapzo [package insert]. Eagle Pharmaceuticals, Inc., Woodcliff Lake, NJ. Available at: <https://belrapzo.com/prescribing-information.pdf>
2. Bendeka [package insert]. Teva Pharmaceuticals. Parsipanny, NJ. Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/208194s026lbl.pdf
3. Treanda [package insert]. Cephalon Oncology. Frazer, PA. Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/022249s026lbl.pdf
4. Vivimusta [package insert]. Slayback Pharma LLC. Princeton, NJ. Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/212209s005lbl.pdf

5. Bendamustine. NCCN Drugs and Biologics Compendium. Available at: https://www.nccn.org/professionals/drug_compendium/content/
6. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®). National Comprehensive Cancer Network, Inc. 2025. All rights reserved. Accessed August 19, 2025. To view the most recent and complete version of the guideline, go online to NCCN.org.

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Date	Updates
11/7/2023	Addition of HCPCS; Addition of Universal Criteria; Updated references
4/25/2024	Annual Review; Universal Criteria language update
9/24/2024	updated PI links; updated to QUMC approved policy template
11/4/2024	Updated HCPCS Codes per CMS 1/1/2025 update – removed J9058, J9059; updated description for J9033
10/1/2025	OH P&T Annual review – added dr reddy product; approved by OH P&T
10/22/25	Policy adopted by GEHA; document reviewed for universal accessibility.